[redacted]

DOCTOR'S ADMISSION NOTE	JIMSH/FORM/DOC/01/Ver 1

Date: 15/10/25 Time:
Name: [redacted]
Age/Sex: M 14 Y
IP Number: IP 25/10/50054
Department/Unit: ~~ONCO SX~~ PES.
Ward / Bed: MSW 2/19

Paste Sticker

CC (With Duration): Swelling in (Lt) side of face 1st unnoticed 4 months ago

HPI:

The Pt was apparently well 4 months back After which he developed a swelling on the lt side of the face. on examination the swelling was 3x4cm on the (Rt) ridge of mandible. HPE was done on 08/08/25 and the diagnosis was squamous cell carcinoma.

K/C/O Type 2 Diabetes mellitus.

FOS:

PMH:

PSHx:

Meds: (Dose, frequency, route)

NO	SH: Tobacco: / NO Alcohol: Drugs:	Allergies: / Not known

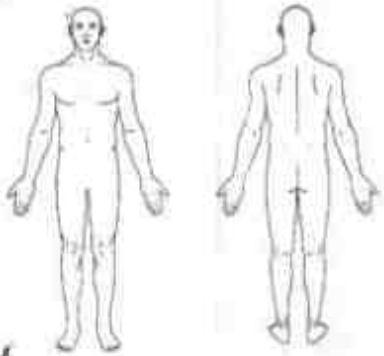
Physical Exam:

Admit Wt.: Height:

BP: 120/80 Pulse: 86/min RR: 20/min Temp: Af SpO2: 98% (RA)

BMI:

- GEN: A/C/C
- HEENT: NAD
- RESP: B/L VBS (+)
- CV: S1S2 (+)
- ABD: Soft, TPS (+)
- EXTREMITIES: / W.N.L.
- NEURO:
- SKIN:
- LOCAL: 3x2 cm solid firm swelling on (Lt) side of the face.



Labs: (Baseline)

WBC — Hgb / HCT — PLT

Na / K — Cl / HCO_3 — Urea / Cr — Glucose

Outside Investigations:

Radiology /ECG / Echo :

Diagnosis/ DD : Squamous cell carcinoma of hard palate.

Treatment/Plan :

Adv

- B/F - CBC, RFT, LFT, HbA1c, PPBS, FBS, PT/INR, TSH, FT4, Serology, ABO.
- Chest x-ray PA view.
- ECG - 12 leads.

Done

- INJ. PAN (40) IV OD AC
- INJ PCM (1g) IV BD SOS
- Check CBG TDS ~ HOLD
- Monitor Vitals -
- Inform SOS
- TAB. SOFTERON - 1 TAB P/O - BD
- INJ. DYNAPER AQ - 1 Amp - IM - SOS

SR / Consultant Note :

[redacted] UHID 86358

Name with Date & Time :

Name with Date & Time :

[redacted]

DAILY PROGRESS NOTE

JIMSH/FORM/DOC/02/Ver 1

Patient Name [redacted] IP No. 2510150059 Dept. DNCO-SX

Date 16/10/25 Time 8:30 am Ward MSW-2 Bed 19

C/S by: PGT Date & Time: 16/10/25

S (Patient's Complaints/ Over Night Events):

pain in the back of neck.

O: Vitals:

Temp: AF HR: 82 BP: 110/70 RR: 20 SpO2: 98% RA

CBG:

In / Out

Physical Exam:

- GEN: A/C/C.
- HEENT: NAD
- RESP: B/L VBS+
- CV: S1S2+
- ABD: SGT (BS+
- EXTREMITIES:
- NEURO:
- SKIN:
- LOCAL:

} WNL

Urine } passed
Stool

Labs:

WBC | Hb / Hct | Plt | Na / K | Cl / HCO3 | Urea / Cr | Glucose

Radiology:

ECG/Echo:

JR Note (Diagnosis / Plan)

Δ → Squamous cell CA of hard palate.

Plt → MRND + Free Flap Reconstruction + Tracheostomy tomorrow 17/10/25.

Pre op

- NPO from Midnight
- AGS + KGD
- OT consent / approval
- PAC to follow
- Inj. TT (0.5) IM stat
- T. ALPRAX (0.5) @ 10pm today

Repeat Hb today

SR/Consultant Note (Diagnosis / Plan):

- Inj. XONTA (4g) APST
- Inj. PAN (40)
- Inj. ONDEM (4)

} IV CM

- IVF NS : RS (1:1) @ 60ml/hr

[redacted]

Name: Time:

Name: Time:

90552

[redacted]

JIMSH/FORM/DOC/02/Ver 1

DAILY PROGRESS NOTE

Patient Name: [redacted] IP No. 251015005U Dept. PMS

Date: 16/10 Time: 7PM Ward: NSW-11 Bed: 19

C/S by: [illegible] Date & Time:

S (Patient's Complaints/ Over Night Events):

CBG:
In
Out

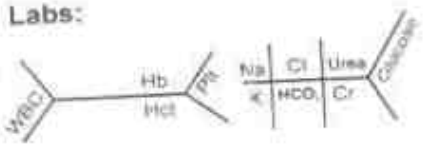
O: Vitals:
Temp: HR: BP: RR: SpO2:

Physical Exam:

- GEN
- HEENT:
- RESP:
- CV:
- ABD:
- EXTREMITIES:
- NEURO:
- SKIN:
- LOCAL:

ICU and venti consent

Labs:



Radiology:

ECG/Echo:

JR Note (Diagnosis / Plan)

MR. WARIS AZAM 40 year male patient was admitted in MSW2/19 with diagnosis of Squamous cell carcinoma of hard palate will undergo MRND + free flap reconstruction + Tracheostomy under GA on 17/10/2025., may require for ICU care + ventilatory support for better management after surgery

Nephew

Md Sufiyan

मोनी (Son)

16-10-25

Name: Time: 7PM

SR/Consultant Note (Diagnosis / Plan):

Name: Time:

[redacted]

DAILY PROGRESS NOTE	JIMSH/FORM/DOC/02/Ver 1

Patient Name: W. Amis: A2 Am IP No: 25/0/5105 Dept: PRS

Date: 17/10/25 Time: 1.30pm Ward: MSW Bed: 19

C/S by: SR Date & Time: 17/10/25 (m)

S (Patient's Complaints/ Over Night Events):
L/O fresh complaints.

O: Vitals:
Temp: Afb HR 80/p BP 110/80 mmHg RR 20/min SpO2 98% RA

CBG:
In 3000 ml
Out 2100 ml

Physical Exam:
- GEN: ACC
- HEENT: NAD
- RESP: B/L, NBS+ve
- CV: S1S2 heard
- ABD: SOFT, BS+ve
- EXTREMITIES:
- NEURO:
- SKIN: } WNL
- LOCAL: } 7x2 cm solid swelling, on left side of face.

Labs:
WBC | Hb / Hct | Plt | Na / K | Cl / HCO3 | Urea / Cr | Glucose

Radiology:

ECG/Echo:

JR Note (Diagnosis / Plan)	SR/Consultant Note (Diagnosis / Plan):
DSCC of Hard palate.	
17/10/25 1.30P.	Case seen by Dr A.K. Jha.
I/D Cw Dr. Tapan Haldar (PGT) Plan PRS.	Mucopolyferative growth left maxilla, hard palate
- NPM over	
- OT postponed - advanced Ca.	OTC - Hard mass involving the right maxilla, mandible
- Inf SVT	ad-
	- Semisolid diet
	- Ref to medical oncologist for opinion
	- Pt [redacted] for my sick.
Name: [redacted] Time:	Name: [redacted] 1.30 pm

DR. AMITESH KUMAR JHA
MBBS, MD
Asst Prof
Reg. No.

[redacted]

DAILY PROGRESS NOTE | JIMSH/FORM/DOC/02/Ver 1

Patient Name : [redacted] IP No. 2510150054 Dept. PRS

Date : 18/10/25 Time : 9:10 AM Ward : NW2 Bed : 19

C/S by: GS - PGT Date & Time:

S *(Patient's Complaints/ Over Night Events)*:

~~Pain~~ ~~no fresh c/o~~
throat pain,

O: Vitals:
Temp: AF HR: 75 BP: 120/70 RR: 18 SpO2: 99% @ RA

CBG:
In
Out

Physical Exam:

- ☐ GEN: afuc
- ☐ HEENT:
- ☐ RESP: b/L vBS ⊕
- ☐ CV: S1S2 ⊕
- ☐ ABD: soft, nt, BS ⊕
- ☐ EXTREMITIES:
- ☐ NEURO:
- ☐ SKIN:
- ☐ LOCAL:

} WNL

urine ✓
stool ✓

Labs:

WBC | Hb / Hct | Plt | Na / K | Cl / HCO3 | Urea / Cr | Glucose

Radiology:

ECG/Echo:

JR Note (Diagnosis / Plan)	SR/Consultant Note (Diagnosis / Plan):
D: Squamous cell ca of hard palate	
P:	
- Refer to Oncology	
- Cont. rst Sue.	
[redacted] Time: 11 AM	Name: Time:

[redacted]

CROSS CONSULTATION / REFERRAL NOTE

JIMSH/FORM/DOC/04/Ver 1

Date : [redacted]
Name : [redacted]
Age/Sex : M/19
IP Number : 25/015 0054
Department/Unit : PRS-U1
Ward / Bed : MSW-2/19

Paste Sticker

Consult Department: Refer to Oncology.

Reason for consultation *(To be filled by consulting department)*:

HPI: 40 yrs old M/W was Azam admitted under Dr. A.K. Jha presented with squamous cell carcinoma of hard palate. Please examine the patient and give your valuable opinion and transfer under your care for further management. [redacted]

PH / PSH / FH / SH:
☐ Same as admission note except for:

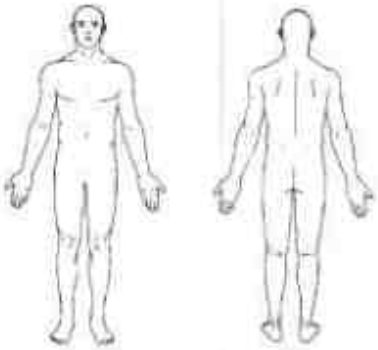
Present Medication: (Dose, frequency, route)
☐ Same as admission except for

Physical Exam :

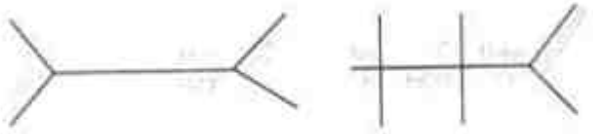
BP: Pulse: RR: Temp: SpO2 : Admit Wt.: Height : BMI :

- GEN:
- HEENT:
- RESP:
- CV:
- ABD:
- EXTREMITIES:
- NEURO:
- SKIN:
- LOCAL:

[redacted]



Labs: (Baseline)



Other Labs:

Radiology / ECG / Echo:

Diagnosis:

Treatment/ Plan:

Locally advanced
Gen Maxilla (B)
c̄ bilat neck nodes

Adv: Surgery
Neoadj chemo
~3 cycles
↓
Biopsies
↓
[redacted]

Signature with Date & Time: 18/10/25 at 2:20pm

Name:

[redacted]

CROSS CONSULTATION / REFERRAL NOTE

JIMSH/FORM/DOC/04/Ver 1

Date : 18/10/25 Time :

Name : [redacted]

Age/Sex : 40Y/M

IP Number : IP251015 0054

Department/Unit : Oncology

Ward / Bed :

Paste Sticker

Consult Department: OMFS (Oral Oncology)

Reason for consultation (To be filled by consulting department):

CASE [redacted]

HPI: KCO biopsy proven SCC of palate & lower gum

c/o LT neck mass

Ulcerative lesion +

FNAC - pending

inoperable

PMH / PSH / FH / SH:

☐ Same as admission note except for:

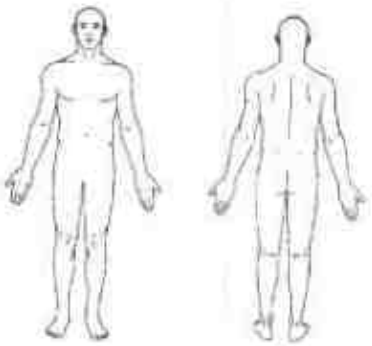
Present Medication: (Dose, frequency, route)

☐ Same as admission except for

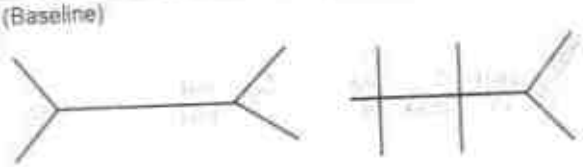
Physical Exam :

BP: Pulse: RR: Temp: SpO2 : Admit Wt.: Height : BMI :

- GEN:
- HEENT:
- RESP:
- CV:
- ABD:
- EXTREMITIES:
- NEURO:
- SKIN:
- LOCAL:



Labs: (Baseline)



Other Labs:

Radiology / ECG / Echo:

Diagnosis: Voice change → probable involving vocal cord

Extensive clinically.

Treatment/ Plan:

At present, Difficult to arrive to margin.

Refer to MRI Oncologist for consideration of Chemotherapy / NACT

&

Assess.

[redacted]

Signature with Date & Time: 18/10/25

Name:

[redacted]

DAILY PROGRESS NOTE	JIMSH/FORM/DOC/02/Ver 1

Patient Name : [redacted] IP No: 2510150054 Dept: PRS

Date : Time : Ward : MSW2 Bed: 19

C/S by: GS Date & Time: 16/10/25

S *(Patient's Complaints/ Over Night Events)*:

O: Vitals:
Temp: AF HR: 80/- BP: 120/80 RR: 19 SpO2: 99%

CBG:
In
Out

Physical Exam:

- GEN: Acc
- HEENT: nn
- RESP: mn
- CV:
- ABD: Sr.
- EXTREMITIES:
- NEURO:
- SKIN:
- LOCAL:

Labs:

WBC | Hb / Hct | Plt | Na / K | Cl / HCO3 | Urea / Cr | Glucose

Radiology:

ECG/Echo:

JR Note (Diagnosis / Plan)	SR/Consultant Note (Diagnosis / Plan):
A Squamous cell cancer of hard palate (Locally advanced Ca maxilla (left))	
- Follow oncology advised.	
- Hold Oratin DS	Patient relative (brother in law) explained details about patient condition and OT postponed matter.
Tab. Calpol T 1 tab SOS.	
211 d	

[redacted]

Name: Time:

[redacted]

DAILY PROGRESS NOTE | JIMSH/FORM/DOC/02/Ver 1

Patient Name [redacted] IP No: 251015 0054 Dept: PPG

Date: 19/10/25 Time: Ward: MS-2 Bed: 12

C/S by: PGI Date & Time:

S (Patient's Complaints/ Over Night Events):

No fresh c/o.

O: Vitals: Temp: AF HR: 70 BP: 170/70 RR: 22/min SpO2: 98% (RA) CBG: In / Out

Physical Exam:

- GEN: Mclc
- HEENT:
- RESP: B/L vBS (+)
- CV: S1 S2 (+)
- ABD: K.T. (+) BS (+)
- EXTREMITIES:
- NEURO: CNM.
- SKIN:
- LOCAL:

Stool / urine passed.

Labs:

WBC | Hb / Hct | Plt | Na / K | Cl / HCO3 | Urea / Cr | Glucose

Radiology:

ECG/Echo:

JR Note (Diagnosis / Plan)	SR/Consultant Note (Diagnosis / Plan):
Δ Squamous Cell CA of Hard Palate. (locally adv. maxillary CA)	
D	
– Cat-all.	
– Inform s/s.	
– IVF DNS over 6hr (1 bottle)	
[redacted]	
Time: 11:50AM	Name: Time:

[redacted]

DAILY PROGRESS NOTE | JIMSH/FORM/DOC/02/Ver 1

Patient Name: [redacted] IP No: 25/015 0054 Dept: PRS - D

Date: 20/10/25 Time: M 10 am Ward: MSW Bed: (19)

C/S by: Date & Time:

S (Patient's Complaints/ Over Night Events):

No pain in neck region

O: Vitals:

Temp: AF HR: 90/m BP: 120/70 RR: 20 SpO2: 98% in RA

CBG: In 2600 ml Out 2450 ml

Physical Exam:

- GEN: A/c/c
- HEENT:
- RESP: B/L VBS (+)
- CV: SS2 (+)
- ABD: Soft IPS (+)
- EXTREMITIES:
- NEURO:
- SKIN:
- LOCAL:

WNL

Labs:

WBC | Hb / Hct | PT | Na / K | Cl / HCO3 | Urea / Cr | Glucose

Stool passed

Urine passed

Radiology:

ECG/Echo:

JR Note (Diagnosis / Plan)

A Squamous cell CA of Hard Palate (Locally adv maxillary CA)

20/10/25 12-30 pm

IV FDNS:RL (1:1) @ 8 hrs

If pt is not transferred to oncology then the pt can be discharged tomorrow & referred to higher centre

[redacted]

Name: Time:

SR/Consultant Note (Diagnosis / Plan):

Case. Review Dr A... pt

Locally Advanced carcinoma of hard palate

- pt newly medicalised by plan - Neoadjuvant chemoradiotherapy followed by consolidation for resect ...

- Pt can be transferred to oncology ... Neoadjuvant ...

[redacted]

Name:

[redacted]

DAILY PROGRESS NOTE	JIMSH/FORM/DOC/02/Ver 1

Patient Name: [redacted] IP No. 25/0150054 Dept. PRS

Date: 21/10/25 Time: 9:00 am Ward: HSW Bed: 19

C/S by: ..... Date & Time: .....

S (Patient's Complaints/ Over Night Events):

Pain in Right neck

O: Vitals:

Temp: AF HR: 82/min BP: 110/70 mHg RR: 20/min, SpO2: 98% in RA

CBG: In 1400 ml Out 1100 ml

Physical Exam:

- GEN: A/C/C
- HEENT: NAD
- RESP: B/L VB ⊕
- CV: S1S2 ⊕
- ABD: Soft IPS ⊕
- EXTREMITIES:
- NEURO:
- SKIN:
- LOCAL:

} WNL

Labs:

WBC | Hb / Hct | PLT | Na / K | Cl / HCO3 | Urea / Cr | Glucose

Stool ✓

Urine ✓

Radiology:

ECG/Echo:

JR Note (Diagnosis / Plan)	SR/Consultant Note (Diagnosis / Plan):
Δ Squamous cell CA of hard palate Locally adv. maxillary CA Adv. → Cancel 2 unit PRBC → CST.	
[redacted] Name: Time:	Name: Time: